



LABORATORY / DIAGNOSTIC RECORD

Patient and Owner Information

Patient:	Hanon	Owner:	brandon rose
Breed:	Bernedoodle (Canine)	Address:	4829 Lemona Ave, Sherman Oaks, CA 91403
Sex:	Male (neutered)	County:	Los Angeles
Color:	sable	Phone:	323-533-0117
Age:	9 years 3 months 3 weeks	Email:	brandonrose2@gmail.com
Birthdate:	Mar 21, 2017		
Weight:	84.6 Lbs (38.37 kg)		
Chip:			

Summary

Items:	Senior Wellness Profile 1 (Superchem, CBC, T4, UA)
Ordered:	Jul 13, 2026
Received:	Jul 14, 2026
Source:	ANTECH WEST

SuperChem

<u>Test</u>	<u>Result</u>	<u>Reference Range</u>	
TOTAL PROTEIN	6.6	5.0-7.4 g/dL	NORMAL
ALBUMIN	2.8	2.7-4.4 g/dL	NORMAL
GLOBULIN	3.8 (H)	1.6-3.6 g/dL	HIGH
A/G RATIO	0.7 (L)	0.8-2.0	LOW
AST (SGOT)	110 (H)	15-66 IU/L	HIGH
ALT (SGPT)	47	12-118 IU/L	NORMAL
ALK PHOS	157 (H)	5-131 IU/L	HIGH
GGT	3	1-12 IU/L	NORMAL
T. BILIRUBIN	0.2	0.1-0.3 mg/dL	NORMAL
BUN	16	6-31 mg/dL	NORMAL
CREATININE	1.1	0.5-1.6 mg/dL	NORMAL

SDMA	10.9	<14.0 UG/dL	
BUN/CREAT RATIO	15	4-27	NORMAL
PHOSPHORUS	1.4 (L)	2.5-6.0 mg/dL	LOW
GLUCOSE	121	70-138 mg/dL	NORMAL
CALCIUM	10.5	8.9-11.4 mg/dL	NORMAL
CORRECTED CALCIUM	11.2		
MAGNESIUM	1.6	1.5-2.5 mEq/L	NORMAL
SODIUM	150	139-154 mEq/L	NORMAL
POTASSIUM	3.8	3.6-5.5 mEq/L	NORMAL
NA/K RATIO	39 (H)	27-38	HIGH
CHLORIDE	115	102-120 mEq/L	NORMAL
CHOLESTEROL	330 (H)	92-324 mg/dL	HIGH
TRIGLYCERIDE	146	29-291 mg/dL	NORMAL
AMYLASE	580	290-1125 IU/L	NORMAL
PrecisionPSL	182 (H)	24-140 U/L	HIGH
In dogs with appropriate clinical signs, this mildly elevated PrecisionPSL result is equivocal for a diagnosis of pancreatitis. In dogs without clinical signs of pancreatitis, a mild PrecisionPSL elevation is an insignificant finding.			
CPK	491	59-895 IU/L	NORMAL

Complete Blood Count

Test	Result	Reference Range	
WBC	13.2	4.0-15.5 10^3 /mL	NORMAL
RBC	6.7	4.8-9.3 10^6 /mL	NORMAL
HGB	16.2	12.1-20.3 g/dL	NORMAL
HCT	50	36-60 %	NORMAL
MCV	75	58-79 fL	NORMAL
MCH	24.3	19-28 pg	NORMAL
MCHC	32	30-38 g/dL	NORMAL
Platelet Count	217	170-400 10^3 /mL	NORMAL
Platelet Estimate	Adequate		
Neutrophils	81 (H)	60-77 %	HIGH
Bands	0	0-3 %	NORMAL

Lymphocytes	15	12-30 %	
Monocytes	3	3-10 %	
Eosinophils	1 (L)	2-10 %	
Basophils	0	0-1 %	
Absolute Neutrophils	10692 (H)	2060-10600 /mL	
Absolute Lymphocytes	1980	690-4500 /mL	
Absolute Monocytes	396	0-840 /mL	
Absolute Eosinophils	132	0-1200 /mL	
Absolute Basophils	0	0-150 /mL	

T4

Test	Result	Reference Range	
T4	2.2	0.8-3.5 mg/dL	

Diagnosis of canine hypothyroidism: for dogs with consistent clinical signs and without non-thyroidal illness, a low-to-low normal T4 concentration (e.g.,

Urinalysis-Complete

Test	Result	Reference Range	
Collection Method	Not Stated		
Color	Dark Yellow		
Appearance	TURBID		
Specific Gravity	1.060 (H)	1.015-1.050	
pH	6.5	5.5-7.0	
Protein	3+ (H)	NEGATIVE	
Urine protein:creatinine ratio testing is recommended (if the sediment is inactive) to help determine the clinical significance of proteinuria.			
Glucose-Strip	NEGATIVE	NEGATIVE	
	Result Verified		
Ketones	NEGATIVE	NEGATIVE	
	Result Verified		
Bilirubin	2+ (H)	NEG TO 1+	
Occult Blood	2+ (H)	NEGATIVE	
WBC	0-1	0-3 HPF	
RBC	11-20 (H)	0-3 HPF	
Casts	NONE SEEN	Hyaline 0-3 LPF	
Calcium Oxalate Dihydrate Crystals	0-1	HPF	
Amorphous Crystals	0-1	HPF	
Bacteria	NONE SEEN	None Seen HPF	
Squamous Epithelia	0-1	0-3 HPF	
Fat Droplets	>50	HPF	

KeyScreen GI Parasite PCR Panel

<u>Test</u>	<u>Result</u>	<u>Reference Range</u>
Ancylostoma spp.	Undetected	
A. caninum resistance marker	Undetected	
Uncinaria stenocephala	Undetected	
Toxocara spp.	Undetected	
Toxocara canis	Undetected	
Toxocara cati	Undetected	
Toxascaris leonina	Undetected	
Baylisascaris procyonis	Undetected	
Trichuris spp.	Undetected	
Giardia duodenalis	Undetected	
Giardia Zoonotic	Undetected	
Cryptosporidium canis	Undetected	
Cryptosporidium felis	Undetected	
Cystoisospora spp.	Undetected	
Eimeria spp.	Undetected	
Dipylidium caninum	Undetected	
Echinococcus multilocularis	Undetected	
Echinococcus granulosus	Undetected	
Taenia spp.	Undetected	
Neospora caninum	Undetected	

Comment

A DETECTED KeyScreen GI Parasite PCR result in a patient with clinical signs that are appropriate to the organism, suggests this is the likely cause of the clinical signs. In the absence of clinical signs, parasite detection could suggest a subclinical infection or be related to coprophagia. Subclinical infection may need to be treated in cases where the parasite is zoonotic, has the potential to cause clinical signs or where continued shedding contributes to environmental contamination. An UNDETECTED KeyScreen GI Parasite PCR result indicates that no parasitic organism was detected. An undetected PCR result most often indicates absence of infection but might also occur after successful treatment or with spontaneous resolution of infection. Undetected results due to cyclical shedding may be overcome with repeat testing or by testing pooled samples collected over multiple days. For infections with an extra-intestinal phase (e.g., echinococcosis, toxoplasmosis, neosporosis), an undetected KeyScreen GI Parasite PCR result does not rule out systemic infection. If systemic infection is suspected, additional diagnostic investigation is indicated. As a reference, we have provided links to CAPC guidelines. CAPC is an independent, non-profit organization. Veterinarians: If the KeyScreen GI Parasite PCR result does not explain the clinical signs or if you require additional interpretive assistance, consultation with an internist is available free of charge (Monday to Friday 8am to 8pm EST, Saturday 9am to 6pm EST) at 1-888-838-4636.